Thinnr Protocol® Protocol Knowledge Base

How Does Homeopathy work?

Homeopathy is based on a process that is called a multi-dilution and succination. Basically, they start with the straight raw products and dilute them. After being diluted, they go through a process that amplifies the energy signature of the molecule. This process is repeated over and over until reaching the final dilution. Each time they dilute, they amplify. This is only speaking of our product, but after the dilution and succination process, the amount of Thinnr Protocol is about 1 ounce of Thinnr Protocol in a 20,000-gallon swimming pool.

THINNR PROTOCOL PROGRAM QUESTIONS

Are there any side effects? Because of the dilute nature of homeopathic medications, side effects and drug interactions are highly unlikely.

Are there any contraindications or any types of people who should not do the Thinnr protocol?

An individual who is experiencing or has experienced the following are prohibited from doing the Thinnr protocol: Women who are pregnant or breastfeeding, any individual who has active cancer (a person who is in remission can do the Thinnr Protocol. Recommend oncologist opinion prior to starting), any individual who has experienced active cholecystitis within the last 4-6 weeks, any individual with what is deemed to be a significant degree of cardiac compromise, an individual has undergone a recent surgical procedure and has not yet been released from the surgeon, and a person who already has a BMI of 25 or lower.

Why Not 3 Meals per Day? This concept is rooted in the proven benefits of intermittent fasting, which can positively influence the hypothalamus—a key regulator of metabolic processes, hunger, and energy balance. The hypothalamus produces hormones that control appetite and stress responses. Intermittent fasting activates pathways in the hypothalamus that respond to nutrient availability, enhancing sensitivity to hormones like insulin and leptin, which are essential for managing hunger and energy use. It also stimulates the production of brain-derived neurotrophic factor (BDNF), a protein linked to improved brain function and neuroprotection.

Does someone take more or less product with more or less weight to lose? No, the dosage remains the same because homeopathic medications do not require that dosing be based on patient size, weight, or sex.

Is It Wrong to Do Thinnr Protocol for Less Than 42 Days? No, but a patient needs 21 days on the program (minimally) to reset set points.

Why the Thinnr Metabolic Activation Phase and why and how does it work? The purpose of the Thinnr Metabolic Activation Phase is to cause the body to increase its metabolism to process the significantly higher caloric and fat intake while the Thinnr Protocol takes effect.

When the patient transitions to the Thinnr Metabolic Thinning the Thinnr Protocol nutritional support formula will help keep the metabolism elevated.

the Thinnr Metabolic Activation Phase The Thinnr Metabolic Activation phase is important and works because of dietary thermogenesis and the dietary thermic effect. The dietary thermic effect accounts for 10-15% of daily caloric expenditure. Thinnr Metabolic Activation phase increases metabolism (burns more calories) to process/digest the increased food intake. The ingestion of nutrients causes an obligatory increase in heat production by the body due to the energy requirements (metabolic costs) of digestion, absorption, metabolism, etc. Additionally, we encourage the Thinnr Metabolic Activation phase to be performed with the use of higher consumption of high-fat protein when clinically appropriate. This is because the thermogenic requirements needed for protein digestion are higher than that of carbohydrates. The first rise in metabolic rate that occurs in response to the presence of food is called the cephalic phase, and the second is called the postprandial phase. The postprandial phase may last up to 6 hours and is exactly why we want the patient to "graze" during the Thinnr Metabolic Activation phase, as it helps keep the increase sustained and helps prevent the "peaks and valleys" that are experienced when a person eats 1-3 times per day. Additionally, the number of meals consumed each day also affects dietary thermogenesis since an increase in the number of meals consumed per day causes an increase in the total amount of heat produced each day. We should also not forget Adaptive Thermogenesis which is an additional energy expenditure that is not included in dietary thermogenesis. Adaptive thermogenesis is manifested primarily as an increase in BMR due to environmental stresses, one of which is an alteration in traditional food intake. Additionally, think of this as an added "bonus" for some people... Overconsumption affects thermogenesis. When energy intake (calories) increases above normal levels needed for dietary thermogenesis and regulation of body temperature. The increase in energy loss is a result of less efficient use of food calories and is thought to represent the body's tendency to protect the status quo of energy balance during periods of overconsumption.

High-calorie grazing all day is imperative because a person who only consumes 3 meals per day will experience peaks and valleys in their metabolism. Grazing prevents these peaks and valleys and as the Thinnr Protocol nutritional support formula may take effect at different times for each patient, it is best for it to take effect at a peak, not during a valley.

What window of time should a person eat? During the Thinnr Metabolic Thinning phase, the patient should consume all food and calories within a 5–7-hour window of time.

Can free foods be consumed as snacks between meals? No. Snacking between meals is prohibited, whether the foods consumed are free or not.

Can a patient exercise while on Thinnr Protocol? It is important to note that exercise is not a required component of the THINNER program. If your patient still wants to exercise, it is recommended that they only do Thinnr Metabolic Thinning Phase approved exercises as outlined in the instruction manual.

If you have a patient that is going to work out during the Thinnr Metabolic Thinning Phase, make them aware of the information above and then share the following recommendations with them

- Limit exercise to 3-4 days per week with the other days being walking or light yoga.

- On training days, avoid any and all intense aerobic exercise.
    - While in the Thinnr Metabolic Thinning Phase, patient will have lower glycogen stores, which will easily be depleted in 15-20 minutes of intense exercise leading to increased likelihood of exercise induced injury and muscle wasting.
- Focus on low to moderate strength training.
    - Do not go above Zone 2 heart rate
    - To estimate heart rate zones, you can use the following general formula.
        - Zone 1 Calculation: (220 – your age) x 0.6
        - Zone 2 Calculation: (220- your age) x 0.7
    - On training days, increase lean protein intake to offset the caloric burn from exercise.

Because exercise is key to long-term weight maintenance and overall health, patients are encouraged to resume their previous routines or begin a more intensive program once they transition into the THINNER Metabolic Normalization phase.

What is the lowest BMI that a person should have to do Thinnr Protocol? The Thinnr Protocol program should only be offered to individuals with a BMI greater than 24 and who do not have any other contraindications to the program.

What are the indicators that the formula is working and not just changes in diet, making things easier for them? If changes in diet were the effective factor, then everyone who cleaned their diet would be fine. Clearly, this is not the case. The ingredients do what they do. It is not a matter of opinion. The HPA is abnormal in most Americans and this is the cause of many health problems. It has been researched and is not open for discussion.

How long do patients take this formula before they agree it is working? They will see results in as little as 4 days. Most patients will start feeling higher energy, clearer thoughts, better sleep, and fewer symptoms in 7-9 days.

What is the Thinnr Metabolic Normalization Phase? Once a person has completed metabolic normalization, they should continue to eat a cleaner diet. Since the setpoints have been changed, the typical person will not gain the weight back very easily unless they return to poor dietary habits.

What can my patient expect during the Thinnr Metabolic Normalization Phase? The person's weight will swing dramatically for the first few days. This is of no concern. As time goes by, the swings will be minimized. We know the new setpoints for BMR and BMI are established when the person's weight is stabilized within 0.5 pounds of itself for 5-7 consecutive days. This is when a person is good to start a 2nd round if needed. Globally, the average person loses an additional 2-4 pounds within the 3-5 weeks it takes for the body weight to stabilize.

The patient gets full and can't eat all of the metabolic normalization portions. What should I say? The patient's appetite will increase when the Protocol formula is discontinued and exercise starts or resumes. The patient should listen to their body and eat when hungry but not "overeat" just to fulfill the "numbers". Just like a patient needs to learn how to make good food choices, they also need to learn when to stop eating. When the patient's weight stabilizes, they can add in healthy fats which will add to the maintenance caloric intake.

Is it okay for a person to add healthy fats during maintenance? Yes, and it is actually encouraged and recommended. During the metabolic normalization phase, patients will continue to eat off the Thinnr Nutrition Strategy Approved Foods list. If a patient would like to start adding in healthy fats, we suggest they do it gradually. They can begin using small amounts of oils for cooking or use small amounts of oil in a homemade salad dressing a couple of times a week. Another example of a healthy fat they could introduce would be avocado.

My patient has hypoglycemia. What should I recommend? If a patient experiences issues of hypoglycemia, typically in the morning and within the first few days of the Thinnr Metabolic Thinnr phase. If they continue to report the symptoms associated with hypoglycemia, have them add 3 ounces of Thinnr-approved fruit for breakfast. This is in addition to the fruit and optional bread/grain that is consumed during lunch and dinner. If the hypoglycemia symptoms are in the afternoon or evening, then the person would eat the 3 ounces of Thinnr-approved fruit at that time. If your patient is a medicated diabetic, they may need to consult with their medical doctor to have their medication levels adjusted.

Is Thinnr Protocol a Keto program? Thinnr Protocol is not a keto program.

Should my patient or a person weigh their food before or after cooking? It is recommended that a person weigh their food before cooking.

My patient says they are hungry or having hunger issues: A person should not be hungry or have low energy while on Thinnr Protocol if they are following the eating program, taking the medication consistently, and drinking enough water (80-100 oz per day). A person who is "hungry" is not in starvation mode. One of the things we see is a person will have a growling stomach, which is usually associated with feeling hungry, but when Thinnr Protocol clients step back and look at it, their stomach is growling but not actually hungry. If the patient is following the program as instructed, drinking the recommended amount of water, is 5 or more days into the Thinnr Metabolic Thinning phase, and still reports hunger, then we recommend that they add in Thinnr-approved free vegetables at mealtime.

For a list of free vegetables, please see the Thinnr Protocol Manual or the Thinnr App

My patient is craving sweets: For sweet cravings, recommend clear choice water from Wal-Mart. The tropical fruit flavor is super sweet. You can also put it in a Popsicle mold and freeze it.

My patient is craving salt: We recommend putting the patient on a high-quality electrolyte supplement, giving them an electrolyte replenishing IV, or having them add more salt to their food.

My patient ran out of their Thinnr Protocol medicine: The bottles contain the appropriate amount of medication required for a person to take all Thinnr Protocol doses as instructed. If your patient "runs out" early, they have either spilled it, are taking more than instructed, or are sharing it with someone. If the patient "runs out" somewhere during weeks 3 or 4, you can be assured that they are sharing the formulation with someone else and have split the cost of the program with them.

My patient lost their bottle of Thinnr Protocol - how long do they need to be off of it before they restart? Can they be off the drops before having to start over? If a patient has not taken the Thinnr Protocol for more than 72 hours, they need to stop the protocol for no less than 10 days off and then restart beginning with the Thinnr Metabolic Activation phase.

Does Support contain caffeine? There are minimal amounts of naturally occurring caffeine because of the ingredients. That said, it is so minimal that it is not required to be listed on the ingredients label. For a frame of reference, the amount of caffeine is less than that in an average cup of decaffeinated coffee. Also, please remember that many patients will read "caffeic acid" and think that that is caffeine or a closely related stimulant. It is NOT. Caffeic acid is an antioxidant.

WEIGHT LOSS

Is it common to lose weight from different areas week to week or to lose little weight but have good inch loss? This is all very typical. Everyone will lose fat from different areas at different rates.

What happens if my patient cheats? They will experience a slowing or stall of their weight loss.

What about clients who only want/need to lose 10-15lbs? When the weight loss client reaches their goal weight, they move directly into the maintenance phase, stop taking drops and wrap Thinnr Protocol in a brown paper bag and refrigerate.

My patient has a BMI between 25 and 28 but still feels they want to lose another 10-15 lbs. Can they do Thinnr Protocol again? They will lose weight, but we need to ensure that the ending body weight still falls within healthy BMI for the doctor's liability. The person will lose a little more slowly.

My patient's weight loss has plateaued or stalled. What should I do or recommend? A plateau or "stall" is when the person does not lose any weight (0.0 pounds) for 72 consecutive hours. If this occurs, instruct the patient to do a "protein-only" day.

Here's how to use the "Protein Day":

- A plateau or stall is losing 0.0 pounds for 3 consecutive morning weigh-ins (72 hours without losing ANY weight)
- When this occurs, the patient is most likely cheating or cheating, whether intentionally or accidentally. This simply means that they are not following the Thinnr Protocol program per the instructions.
- If you have confirmed and verified that they are following the program as instructed and that they are not "cheating," then you will have them do a protein-only day. A "Protein Day" is where they eat ONLY 8-9 ounces of protein at each meal. If they weigh the next morning and have lost weight, they go back to normal Thinnr Metabolic Thinning Phase eating strategy. If they didn't lose any weight, then they will do a 2nd protein day.

- If not #1 and #2, they are most likely a poor protein digestors and should be put on a high-quality proteolytic digestive enzyme to aid digestion, absorption, and utilization
- If a person repetitively resumes losing weight on a protein day but then stalls or plateaus again, they are most likely a poor protein digestor and should be placed on a high-quality proteolytic digestive enzyme formulation to aid them in the digestion, absorption, and utilization of the protein sources they are eating.

There are several possible causes of a plateau or stall while on the Thinnr Protocol. They are: 1) the patient is not following the program EXACTLY as in the Thinnr Protocol Manual, 2) they are not drinking enough water, 3) they are a poor digestor and, as a result, are being pushed into starvation where their body is burning lean mass and storing fat, 4) the patient is not varying the foods they eat. If the patient is following the program, drinking enough water, and varying their food types and still plateau or stall, place them on a high-quality proteolytic digestive enzyme.

My patient's weight loss stalled during her menstrual cycle: This is experienced in some female patients and is related to both hormone changes and water retention. The patient is still losing fat, but it is being offset by water retention. Have patients continue to follow the program as instructed, and their weight loss will resume.

Why did my patient's lipid profile look bad after finishing the Thinnr Metabolic Thinning Phase? This is somewhat common as rapid weight loss can temporarily skew the lipid profile in a negative direction. This will self-correct. It is recommended that the patient not have post-Metabolic Thinning phase blood work for 8 weeks after they have completed the Metabolic Thinning phase.

Will my patient lose a lot of muscle mass during the Metabolic Thinning phase? If the participant is following the Metabolic Thinning phase as instructed, there should be minimal to no loss of muscle mass. If the participant is following the protocols and is losing muscle mass, then they are likely a poor protein digester and should be placed on a high-quality proteolytic digestive enzyme to aid in their ability to digest, absorb, and utilize the protein that they are consuming.

THE FOODS

Do All Foods (protein, vegetables, fruit) Need to Be Consumed At Each Meal? Yes!

Patient Does Not Eat Vegetables: Vegetables MUST be consumed! Vegetables can be mixed into a salad or incorporated into the meal instead of eating separately.

Can Multiple Vegetables Be Mixed? Yes, you can mix vegetables.

Is There A Vegetarian Plan? Yes. There are detailed vegetarian variations in the Thinnr Instruction Manual and Thinnr app.

What Foods Are OK to Eat? Foods listed in the Thinnr Nutrition Strategy Approved Foods list are the only foods allowed to be consumed while a person is on the Thinnr Protocol. If it is not on the list, it is not allowed unless it is specifically listed as an integral part of a Thinnr-Approved recipe. Anything with 0 calories is automatically approved.

Can my patient have Sucralose or Dextrose? Both are okay, but it is recommended that you avoid them if possible, as they both have been shown to slow weight loss.

Can a patient drink diet soda? Diet soda is approved as long as it is 0 calories. We recommend diet soda that contains stevia; however, other artificial sweeteners are OK.

Can a person have salad dressing? Yes, but it must be zero calories.

Can a person have mayonnaise? Yes, but it must be zero calories. We recommend the Walden Farms brand.

Can a person have fermented foods? If a food is not listed in the Thinnr Nutrition Strategy approved foods list it is not allowed unless it is 0 calories. As far as "fermented foods" specifically, sauerkraut and any non-sweet pickles are "Approved Free Vegetables" because the commercial preparation of the most publicly available version meets the criteria for being "approved". Typically, when asked this question, the person asking is actually asking if they can have kimchi or drink kombucha. Unfortunately, the answer is "no". This is because the vast majority of commercial preparation is all over the place, and it is impossible to make a "blanket approval." For example, if you tell a person they can have kombucha, they will typically go just grab their favorite brand regardless of whether it meets the requirements or not. Additionally, most people cannot be trusted to review and interpret the nutrition label facts panel on food, even if they say they will or do.

Can a person have yellow mustard? Yes, as long as it is 0 calories.

Can a person have salad dressing? Yes, but it must be zero calories.

Can a person have Kombucha? Most kombucha contains calories so that would be a "no". If they can find a 0-calorie kombucha, they can have it.

Can a person have eggs? Eggs are not approved during the Thinnr Metabolic Thinning Phase. However, egg whites and egg white substitutes are approved. Eggs are highly recommended once the patient has transitioned to the Thinnr Metabolic Normalization Phase.

Will smoking fish on the approved foods list have any effect? Hot smoked fish is approved as it renders the fat out. Cold smoked fish is not approved as the fat is not rendered out.

Can a person have Snapper? See "Thinnr Approved Foods List" in the Thinnr Protocol Manual and Thinnr app.

Can a person have sugar? Sugar is not approved unless specifically called for in a Thinnr-approved recipe.

Can a person eat extra vegetables or are there free vegetables? Free vegetables are listed in the Thinnr-approved foods list and can be consumed ONLY with meals in reasonable quantities. If a person is experiencing hunger and feels like they need more food, they are most likely not following the program as instructed and are either not eating all of the foods or not drinking enough water.

Can a person have Pam, Spray Butter, or Olive Oil in a Misto? Anything 0 calories is automatically approved.

Can a person juice their fruits and vegetables? If the juicer used extract the fiber, then "no. If the juicer or blender does not extract the fiber, then "yes".

How Does Thinnr Determine What Allows A Food To Be Approved? All foods are put through a proprietary matrix. To disclose the methodology would be providing our proprietary information.

Can a serving of meat be replaced with a serving of vegan protein powder? The use of protein powder is reserved for vegetarians and vegans ONLY! Any protein powder that contains 0 carbohydrates, such as certain pea protein powders, are approved. For most, the patient will need to do a double serving to get the correct caloric intake. A patient needs 160-200 calories of protein at each meal minimally. Please note we have observed a slight reduction in weight loss in individuals who use protein powders while in the Thinnr Metabolic Thinning Phase.

Can a person have or Why no healthy fats like nuts, olive, or avocado oil? These are not allowed during the Thinnr Metabolic Thinning phase because the body will metabolize the consumed oils instead of the abnormal fat deposits in the body. The result is that the patient will lose less weight.

My patient asked me if a particular lunch meat is allowed. All protein, including lunch meat, must be at least 93% lean. This means that it must be at least 93% protein. To determine if a protein is at least 93% lean, perform the following calculation:

Step 1: Protein / (Total Fat + Total Carbohydrates + Protein) = "Y"

Step 2: "Y" x 100 = % Protein (Must be equal to or more than 93 to be approved!)



Vitamins During The Thinnr Metabolic Thinning Phase.

Can my patient take Vitamin E? Powder-based Vitamin E is approved.

Can my patient take Vitamin D? Powder-based Vitamin D is approved.

Can my patient take Omega 3 oils? Powder-based Omega 3 supplements are approved.

Can my patient take DHA & EPA vitamins? Powder-based DHA and EPA supplements are approved.

Can my patient take olive leaf extract? Yes, if not oil-based.

Can my patient take collagen supplements? 0-calorie collagen supplements are approved.

Can my patient take colostrum? No, patients cannot take colostrum while participating in the Thinnr Weight Loss Program.

LOTIONS, SUNBLOCK, COSMETICS/MAKEUP

Can A Patient Use Sunblock? Patients can use oil-free sunblock.

Can a patient use baby oil? yes

Can a patient use coconut oil-based lotions? No

Can a patient use products for chapped or dry lips? A patient can use Carmex.

What type of makeup can be used? Dry and mineral-based cosmetics are highly recommended and approved.

MEDICATION INTERACTIONS

Potential Medication Adjustments that Might Be Necessary While on Thinnr Protocol: If you suspect that a patient may require medication adjustments, refer them back to their prescribing provider(s). Tell patients they may need to have their dosages changed as they lose weight.

Do Protocol or Boost have any side effects? Homeopathic products typically have little to no side effects because they use extremely diluted amounts of natural substances, often to the point where only trace elements remain. This high level of dilution means the body isn't exposed to significant chemical doses, reducing the likelihood of adverse reactions. Additionally,

homeopathy aims to stimulate the body's natural healing response rather than override it, making it generally gentle and well-tolerated.

What are the contraindications to Thinnr Boost and Thinnr Protocol? Individuals prohibited from using Thinnr Boost and Thinnr Protocol include Women who are pregnant or breastfeeding, a person with active cancer, and should not be taken by someone who has recently (last 3-4 weeks) experienced acute gallbladder problems.

Can someone who has had lap band surgery or gastric bypass do the Thinnr Protocol program? Yes

My patient had lap band surgery or gastric bypass and can't eat all of the food at mealtime. What should I recommend? Recommend that they eat the food in cycles. For example, have them start with 5-6 oz. of protein. An hour later, have them eat 5-6 oz. of fruit, and 30 minutes after that, have them eat 5-6 oz. of approved vegetables. If your patient cannot eat 5-6 oz of food at a time, have them eat the most oz they can/are allowed to at a time.

Will a woman on birth control pills still see results? Birth control pills should not negatively impact Thinnr Protocol or Thinnr Boost effectiveness.

Can my patient do Thinnr if they take Levoxyl, and will they still see results? Levoyxl should not negatively impact Thinnr Protocol's results.

Can my patient do Thinnr if they take Nasonex, and will they still see results? Nasonex should not negatively impact Thinnr Protocol's results.

Can my patient do Thinnr if they take Hydrocodone, and will they still see results? Hydrocodone should not negatively impact Thinnr Protocol's results.

Can my patient do Thinnr if they take Synthroid or Levothyroxine, and will they still see results? Synthroid and Levothyroxine should not negatively impact Thinnr Protocol's results.

Can my patient do Thinnr if they take beta-blockers, and will they still see results? Beta-blockers should not negatively impact Thinnr Protocol's results. However, if a patient has a cardiac compromise in excess of 50%, they should consult with their cardiologist prior to starting Thinnr Protocol.

Can my patients do Thinnr if they take anti-anxiety medications, and will they still see results? Some patients who have been taking SSRIs or SSNIs consistently for five or more years may experience a slight reduction in program results.

Can my patient do Thinnr if they take Phentermine? A patient taking Phentermine for fatigue issues may be able to wind up reducing the intake as Thinnr Protocol functions as a mild adrenal whip and restorer.

Can my patient do Thinnr if they take fertility medications, and will they still see results? The patient may experience a slight reduction in program results. If the patient gets pregnant, they should immediately stop the program.

Can my patient do Thinnr if they take bio-identical hormones, and will they still see results? Some patients on hormone replacement therapy (HRT) may experience a slower weight loss but will still experience good inch loss.

Can my patient do Thinnr if they take Prozac, and will they still see results? Prozac should not negatively impact Thinnr Protocol's results.

Can my patient do Thinnr if they take OTC stomach medications like Tums, and will they still see results? The patient will be required to take OTC stomach medications that are 0-calorie.

Can my patient do Thinnr if they take Interferon, and will they still see results? Interferon should not negatively impact Thinnr Protocol's results.

Can my patient do Thinnr if they take Capaxone, and will they still see results? Capaxone should not negatively impact Thinnr Protocol's results.

Can my patient do Thinnr if they take Wellbutrin, and will they still see results? Wellbutrin should not negatively impact Thinnr Protocol's results.

Can my patient do Thinnr if they take blood pressure medication, and will they still see results? Blood pressure medications should not negatively impact Thinnr Protocol's results. Patients may need to consult their physician for dosage adjustment as they lose weight.

Can my patient do Thinnr if they take cholesterol medication, and will they still see results? Cholesterol medications should not negatively impact Thinnr Protocol's results. Patients may need to consult their physician for dosage adjustment as they lose weight.

Can my patients do Thinnr if they take diabetes medication, and will they still see results? Diabetes medications should not negatively impact Thinnr Protocol's results. Patients may need to consult their physician for dosage adjustment as they lose weight.

Can my patient do Thinnr if they take Coumadin or other "blood thinner" medications, and will they still see results? Coumadin and blood thinner medications should not negatively impact Thinnr Protocol's results. Patients may need to consult their physician for dosage adjustment as they lose weight. Patients on blood thinners should limit or avoid foods high in Vitamin K.

How long does my patient need to be off of GLPs before starting Thinnr Protocol? It is recommended that a patient wait 7-14 days after discontinuing GLP use before they start Thinnr Protocol.

Can my patient do Thinnr if they are on a steroid, and will they still see results? Steroids can slow weight loss progress but not always. This is typically due to 1 or 2 side effects some people experience while on steroids which are 1) water retention and 2) ravenous hunger.

MY PATIENT IS EXPERIENCING…

My patient is experiencing darkening teeth. According to a dentist… the cause is most likely plaque or tartar that was already there, and the formula may have darkened it a bit. Use whitening toothpaste and/or tooth whitening systems.

My patient is experiencing or reports tongue pain where the patient puts the Protocol or Boost medications, which subsides after an hour or so of application: Pain under or on the tongue is most often related to the patient having a past history of Herpes Simplex 1 (fever blisters). If the patient has a history of fever blisters, have them take an additional L-lysine supplement. It is also recommended that you refer the patient to their medical professional or dentist to have an oral examination.

My patient is experiencing hives/rash: Patients with a rash or hives without nausea is most likely due to detoxing, parasites, or yeast.

My patient is experiencing an acne breakout: Absolutely common as the toxins are being released from the fat cells.

My patient is experiencing night cramping: Have the patient start taking a high-quality electrolyte supplement, or if your office provides IV therapy, consider recommending an IV-based electrolyte replenisher.

My patient is experiencing hair loss. Protein deficiency can cause hair loss. The good news is that it comes back. A small percentage of women could experience mild hair loss if they experience it during pregnancy. Patients experiencing hair loss while on the Thinnr Protocol should up their protein consumption to 1 ounce at each meal. It is also important to consider other causes of hair loss.

My patient is experiencing hypoglycemia symptoms: The only change in protocol is to have the patient eat 3 ounces of Thinnr-approved fruit in the morning to help control blood sugar.

My patient is experiencing low energy or fatigue, which is usually related to low blood sugar or low electrolyte levels. Have the patient start taking a high-quality electrolyte supplement, or if your office provides IV therapy, consider recommending an IV-based electrolyte replenisher. Make sure the patient is drinking enough water. See the section that discusses hypoglycemia for recommendations.

My patient is experiencing insomnia: Very rare side effect. Usually, it will straighten out after 2 weeks.

My patient is lightheaded? This is most likely due to either hypoglycemia or an electrolyte imbalance.

My patient is experiencing spotting or an unexpected menstrual cycle? It occurs in approximately 10-15% of female patients. It is most often peri- and pre-menopausal, but not always. It is related to estrogen being released from the fat cells that are being broken down and evacuated.

My patient missed her period or had a late menstrual cycle? Stop the program immediately and take a pregnancy test. If the pregnancy test is positive, discontinue the program. If the pregnancy test is negative, resume the program.

My patient had a positive pregnancy test? Stop the program immediately!

My patient is having headaches. What should I do? Headaches while on Thinnr Protocol are typically related to detoxification, breaking carbohydrate addiction, and/or low electrolyte levels. Make sure that the patient is drinking enough water and/or have the patient start taking an electrolyte supplement or receive an electrolyte replenisher IV. A detox-type headache is common in 20% of patients on days 3-4.

My patient is constipated. What should I do?

Giving a single solution that fits nearly all patients with bowel motility challenges is unrealistic. However, there are several common denominators among these individuals.

Constipation while on Thinnr Protocol is typically due to several possible reasons: 1) less food being consumed and less filler in the food. This equals less waste produced, 2) low electrolyte levels, 3) the patient is not drinking enough water, and 4) a significant change in the patient's dietary habits. Options to recommend include having the patient begin taking an electrolyte supplement or an electrolyte replenisher IV, having them start taking magnesium, having them take an OTC stool softener, or, depending on how severe the constipation is, a laxative.

If the patient has a pre-existing history of constipation, it is recommended that they start taking a sugar-free fiber supplement beginning on the first day of the Thinnr Metabolic Thinning Phase.

Common causes for constipation during weight loss include: low fiber intake, dehydration, less food intake and "cleaner" food being consumed, electrolyte imbalance, and lack of movement.

To help identify what the cause may be, begin by asking the patient the following questions and check their food tracking to confirm their answers:

1) Are they eating the correct portion of vegetables and fruit at each meal? the combo of the two should contain enough fiber to keep things moving and keep the food intake from being too heavily loaded with protein. If fiber intake is a concern, a person can take an OTC sugar-free fiber supplement.

2) Are they drinking the correct amount of water? Not liquids but actual water. Dehydration during weight loss is a common cause, and the body needs water to get the fiber to work properly in the digestive tract.

3) Are the foods that they are eating a good source of magnesium? Low magnesium is frequently related to slow bowel motility and constipation. Any good quality OTC magnesium supplement can be recommended.

4) Are they doing any form of exercise at all? While not required, a lack of movement can lead to a decrease in intestinal motility.

5) Have they experienced either intermittent or chronic constipation previously in their life? If the patient has a pre-existing history of constipation, it is recommended that they start taking a sugar-free fiber supplement beginning on the first day of the Thinnr Metabolic Thinning Phase.

My patient started unexpected menses while in the Thinnr Metabolic Activation phase or within 3 days of the Thinnr Metabolic Activation phase. What should I recommend? This is less than an ideal situation, but we recommend giving the patient the choice of continuing or stopping for 10 days and then restarting the Thinnr program, beginning with the Thinnr Metabolic Activation phase. If the patient chooses not to stop and restart, tell them that if they do not lose at least 4 pounds in the first week of the Thinnr Metabolic Thinning Phase, they will need to stop for 10 days and start over.

My patient says that they feel cold all the time or like they can't get warm. What should I tell them? This is a somewhat common experience during the Thinnr Metabolic Activation phase. Tell them that while they are on Thinnr, there are several reasons they could be feeling like this and that there is no reason for them to be concerned. Possible reasons include:

1. As they lose fat, their body's "insulation layer" is reduced.
2. not burning as many calories, therefore not as much heat being generated.
3. The liver may not be producing as much alcohol as fatty liver infiltration is decreasing. The liver often becomes less steatotic while on the Thinnr Protocol.

My patient is experiencing bad breath while on the protocol. Why? 'Detox breath' - the lungs are a huge detoxifier of the body. Make sure they are drinking the recommended amount of water daily and that they are eating the proper portions of their fruits and vegetables, or else the body could go into ketosis and cause this 'detox breath' to worsen.

THINNR PROTOCOL USE WITH PATIENTS WITH EXISTING CONDITIONS

Can a patient with heart problems do Thinnr? Patients with severe heart issues should only be placed on Thinnr Protocol if they are strict with the program and in consultation with their medical professional.

Can a patient with diabetes do Thinnr? The Thinnr Protocol is safe for Type I and Type II diabetics. Make the following adjustments. Make sure the patient loads predominantly with high-fat protein.

Can a patient with ovarian or breast cancer do Thinnr? Since the Thinnr Protocol does not contain any hormones, it is safe for women with a HISTORY of breast and ovarian cancer. However, for active cancer or if a person is actively receiving treatment for cancer, participation in the Thinnr Protocol is prohibited.

Can a patient with PCOS do Thinnr? Patients with PCOS can do Thinnr and should experience amazing results.

Can a patient with uterine fibroids do Thinnr? There is no contraindication with Thinnr Protocol for female patients with uterine fibroids. If she is not comfortable, have her check with her OBGYN for an opinion

Can a patient who has had a hysterectomy do Thinnr? The Thinnr Protocol program is fine for a patient who has had a full hysterectomy after being released by the surgeon.

Does this have any long-term adverse effects on someone wanting to have children within the next couple of years? No

If a woman stops breastfeeding, is there a certain amount of time she should wait prior to starting the diet? It is recommended that a woman who has been breastfeeding wait 2-3 months after they have stopped breastfeeding before starting the Thinnr protocol. This recommendation is to allow adequate time for the hormones to balance out and normalize.

Will Thinnr have any effect on menses or menstrual cycles? On occasion, some women may have their menstrual cycle start earlier, last slightly longer, and be slightly heavier while on Thinnr Protocol. In some rare cases, some women have had their menses schedule change but resume the 28-day cycle. For best weight loss success, do not start the Thinnr Protocol program for a female who is within 5 days of starting her menses or until 3 days after the cycle has ended because of hormone fluctuations related to menses.

Can a patient with Lyme Disease do Thinnr Protocol? There are no adverse complications for individuals with Lyme's disease.

Can a patient with Lupus do Thinnr Protocol? There are no adverse complications for individuals with Lupus.

How long does a patient have to wait after surgery to start Thinnr Protocol? As long as the surgeon has told them they are released, the program is okay. This applies to any recent surgery. Document the patient's surgical release date in the patient's file.

Can a patient with Crohn's do Thinnr Protocol? Crohn's is not a contraindication. The Thinnr Nutrition Strategy Approved Foods eliminate many foods known to trigger or exacerbated Crohn's symptoms. However, foods that are high in fiber, raw vegetables, raw fruits, and red meat/pork can also be triggers for some Crohn's sufferers.

Can a patient with Multiple Sclerosis (MS) do Thinnr Protocol? There are no adverse complications for individuals with MS.

Can a patient with Hashimoto's Thyroiditis Lyme do the Thinnr Protocol? There are no adverse complications for individuals with Hashimoto's Thyroiditis.

My patient has had their pituitary gland removed. Can they do the Thinnr Protocol? As long as a patient is not actively in treatment, Thinnr Protocol won't cause any issues. The patient may see a slower weight loss because of the pituitary's role in the HPAT axis, and the patient should be told this right away.

My patient has gallstones or has a past history of gallstones. Can they do the Thinnr Protocol? Gallbladder issues within 4-6 weeks is a contraindication. If it has been more than 4

weeks since their last gallbladder attack, they are cleared to do the program. However, it is HIGHLY recommended that the person be placed on a high-quality gallbladder support supplement, a liver detoxifier, and a high-quality proteolytic digestive enzyme from the outset of the program. If a person has a history of gallbladder issues or has had their gallbladder removed, then they would modify the Thinnr Metabolic Activation phase by limiting foods that are high in fat. On rare occasions when a person is losing fat rapidly, the liver has a hard time processing the cholesterol and triglycerides, and the liver will "spill over" the cholesterol, which can lead to the development of gallstones. It is also a sign that the person has gallbladder stress.

My patient has high B12 levels, or My patient has low B12 levels. Can they do Thinnr Protocol? Yes

My patient has an Iodine/shellfish allergy. Can they do the Thinnr Protocol? There is no iodine in Thinnr Protocol or Boost and no shellfish used in the Thinnr Protocol or Boost formula manufacturing.

My patient has kidney problems? Can they do Thinnr Protocol? There should not be any issues as kidney problems are not a contraindication. However, have the patient consult with their renal specialist before starting. If a person has very significant impaired kidney function such as requiring dialysis or is on the transplant list, the program is not recommended.

My patient needs to take medication twice daily with food. What should they do? Have the patient take their first dose of the medication around 9:00AM with 3 ounces of approved fruit. This will shorten the time of intermittent fasting, but not by much.

My patient has Parkinson's. Can they do Thinnr Protocol? Parkinson's disease is not a contraindication. However, consult with the patient's neurologist before starting Thinnr. That said, there are patients with Parkinson's who have done well and some who have not. From the feedback we have received, the ones who have not done well experienced a temporary increase in their Parkinson's symptoms. Our recommendation is that if you choose to have a patient who has Parkinson's on Thinnr Protocol, be upfront with them. Additionally, and while not protocol-centric, you may also choose to have the patient take high-quality EFAs during the Thinnr Metabolic Thinning Phase as the benefits of the person losing weight will outweigh the negative weight loss impact of them taking the EFAs.

My patient has Pancreatitis. Can they do Thinnr Protocol? Pancreatitis is not a contraindication for Thinnr Protocol. However, because there are many causes of pancreatitis, with the most common causes being excessive alcohol consumption, alcoholism, and gallstones, you should always speak with the patient to determine the etiology. If the patient does not have either of these, it is recommended that you refer the patient to their medical provider as, oftentimes, there are underlying pathology that may be the cause.

If the patient has a past history of pancreatitis or is experiencing an active case, they can do Thinnr Protocol as the Metabolic Thinning Phase type of food consumption can be beneficial. That said, since gallstones are one of the leading causes of pancreatitis, and while we are not the patient's provider and cannot make direct recommendations, you should strongly consider having the patient take a high-quality gallbladder support formulation and from the outset of the

Thinnr Protocol. If the patient has a history of gallstones or has had their gallbladder removed, you should recommend that they follow the "gallbladder modification" for the Thinnr Metabolic Activation phase days.

My patient has MTHFR. Can they do Thinnr Protocol? Yes. It would be strongly recommended that you ask your patient if they are currently taking any supplements with 5-MTHF and methylcobalamin. If they are not, then it is strongly recommended that you recommend that they take them while on Thinnr Protocol. These supplements are important because MTHF is what the body utilizes and comes from the breakdown/conversion of folate and folic acid. A person with MTHFR has a genetic defect that prevents their body from being able to perform the conversion. Methylcobalamin is also important as one of its functions is as a cofactor for the enzyme methionine synthase, which is responsible for converting homocysteine to methionine. Additionally, if needed, you can shift the intermittent fasting to a 14-10 schedule.

FINANCIAL QUESTIONS

Is or can the Thinnr Weight Loss Program be covered by a patient's H.S.A or F.S.A? Most patients with HSAs and FSAs will be eligible for reimbursement.

Can discounts and referral fees be offered? We are not healthcare attorneys and are not giving legal advice. That said, based on consultation with our healthcare compliance experts, we have been told "Yes" as Thinnr Protocol is considered a non-reimbursable service. However, you should speak with your own healthcare attorney or compliance consultant to receive an answer specifically for your business.

My patient requires multiple rounds, How much should I charge? We recommend discounting each Thinnr cycle by $150-$200 for multiple rounds or cycles.

MISCELLANEOUS QUESTIONS

My patient spilled their bottle of Thinnr Protocol. What should I do? It is recommended that you charge the patient for a new bottle. That said, it is also recommended that you tell the patient upfront that if they spill or lose their bottle, they will be charged for another bottle. What you charge for the replacement bottle is at your discretion.

Can a person who maintains a kosher lifestyle do Thinnr Protocol? Having consulted with multiple rabbis, including an orthodox rabbi, a person who maintains a Kosher lifestyle can undergo the Thinnr Protocol Weight Loss Program even though the product itself is not kosher-certified. Here is what we were told:

An individual who maintains a kosher lifestyle can consume the Thinnr Protocol nutritional support formula without violating the tenants because consumption of non-kosher items is permitted when it is for the purpose of treating a health condition that: 1) is life-threatening or 2) if left untreated, has a high probability of being directly related to the development or progression of a health condition that could potentially be life-threatening. Additionally, they all likened it to a person who "keeps kosher," taking medication for a health condition that is not kosher certified.

Here is the permitted reason for consuming non-kosher products: If one is in danger of losing a limb, mobility, eyesight, suffering the loss of some other function vital to one's future survival, or may be in danger of a new health problem developing that may ultimately shorten one's life, the situation is considered to be life-threatening.

How soon after Thinnr Protocol can my patient get blood work performed? A patient should not have routine, non-emergent, or non-urgent blood work done any earlier than 8 weeks post Thinnr Protocol. Fast weight loss stresses the body, and this causes the blood work to come back a little "wonky" sometimes although this is not the case long term.

How soon after Thinnr Protocol can my patient get a hormone panel done? Ideally, a patient will wait 8-12 weeks after they have entered the Thinnr Metabolic Normalization Phase before having a hormone panel performed in order to allow several full hormone cycles for hormone stabilization.

My patient has surgery scheduled or has to have emergency surgery; what should I do or what should I tell them?

It’s recommended that patients with an upcoming surgery wait to start the Thinnr Protocol until after the procedure unless they can complete 21 days of the Thinnr Metabolic Thinning Phase beforehand. This allows for a solid foundation in the metabolic thinning phase before needing to pause. In the event of an emergency surgery, patients should discontinue the protocol to focus on healing and wait until their surgeon clears them to resume.

Why do all patients get the same amount of food during the Thinner Metabolic Thinning Phase? The person’s body will make up the caloric deficit from its fat reserves, which is why heavier people lose more weight. The larger a person is, the higher their BMR (calories burned at rest). The larger the caloric deficit, the more the fat is burned to produce the calories that the person needs.